جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : جيهان عاطف محمد مسعود
المحافظة : الاسكندريه **الحي / المدينة :** الهانوفيل **العنوان :** 0
رقم البطاقة : 27807088800225 **رقم التليفون :** 01211000825 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 08-07-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جيهان عاطف محمد مسعود
National ID: 27807088800225

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

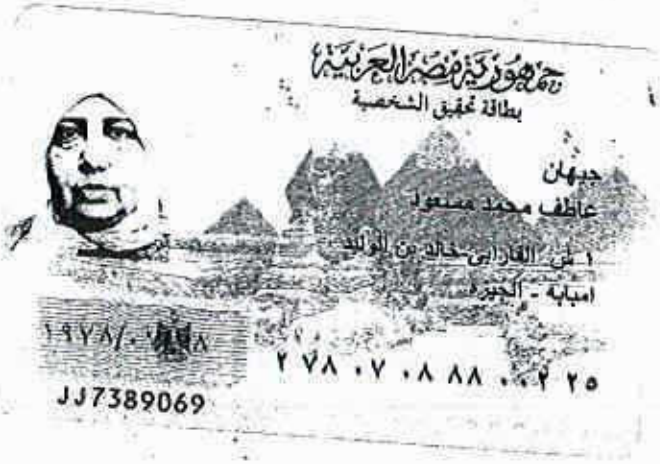
المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


٢٧٨٠٧٠٨٨٨٠٠٢٢٥ ٢٠٢٢/٠٥
حاصله على بكالوريوس خدمه اجتماعيه
انثى مسلمة متزوجة
رامى عنتر رزق احمد
البطاقة سارية حتى ٢٠٢٩/٠٥/٢١

٣,٩٩

زوجيا


جمهورية مصر العربية
بطاقة تحقيق الشخصية
جيهان
عاطف محمد مسعود
١٠ ش الفارابى خالد بن الوليد
امبابة - الجيزة
١٩٧٨/٠
٢٧٨ ٠٧ ٠٨ ٨٨ ٠٠ ٢ ٢٥
JJ7389069

Patient's Name:	[illegible]
Date:	2026 01 31
Referred by:	Dr. [illegible] CLCAP
Hospital:	Gonstave Roussey International Egypt

MULTISLICE NON CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

FINDINGS:

Clinically reported breast carcinoma.

Compared to the last study dated 18/9/2025 ,the current study revealed .

(I) Chest:

- Right breast lesion with nipple retraction .
- No appreciable changes regarding the left axillary lymph nodes . the largest measures 18 x 10 mm compared to 16 mm .
- No sizable pulmonary nodules , masses or cavitataing lesions .
- Still noted enlarged mediastinal lymph node enlargement . the largest at right paratracheal group meaures 16 x 14 mm .
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density no hepatic focal lesions .
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Bilateral renal stones .
- Left mild dilatation of left pelvicalyceal system and left ureter upon lower ureteric stone 8mm with density 650 HU.

CT Scan
[[DOCTYPE:CT_SCAN]]

Normal CT appearance of the spleen , aorta and IVC .

Unremarkable other pelvic organs.

No retro-crural, para-aortic or pelvic lymphadenopathy.

No ascites.

Still noted diffuse scattered sclerotic lesion of axial and appendicular skeleton .

D10 mild vertebral structural collapse .

OPINION:

Overall stationary course of study

Dr.Reham Khedr, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

04/20/2026

MDT FORM

Name:	Jihan . Mohamed	**MRN:**	3099
Age:	48 Y	**ID:**	27807088800225
Menopausal Sta		**Phone:**	01285701974
NRS SCORE:		**Marital Status:**	married
BMI:		**Family History:**	
CO Morbidity:	HTN	**Reason Of MDT:**	

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** +3 **KI67:**

Sonomammography:

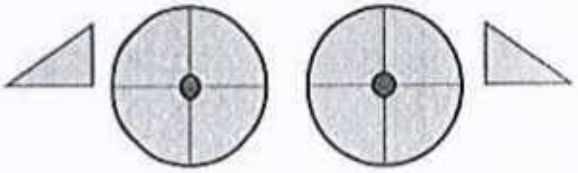
Mets Workup:
2022:BC NACT (AC/TXT)Rt BCS ypT2N PORT tamoxifen
2023: local recurrence.. WLE then received taxol /Carb? x18wk then zoladex +AI
09/2025 .Local RECURRENCE, triple positive , pet: bone mets
received 9 cycles TXT + herceptin IV
FOR AI+ maintainace herceptin IV

History:

Recommendations:
FOR AI+ maintainace herceptin IV

MDT Outcome & Plan
AI+ ZOLADEX +maintainace herceptin IV

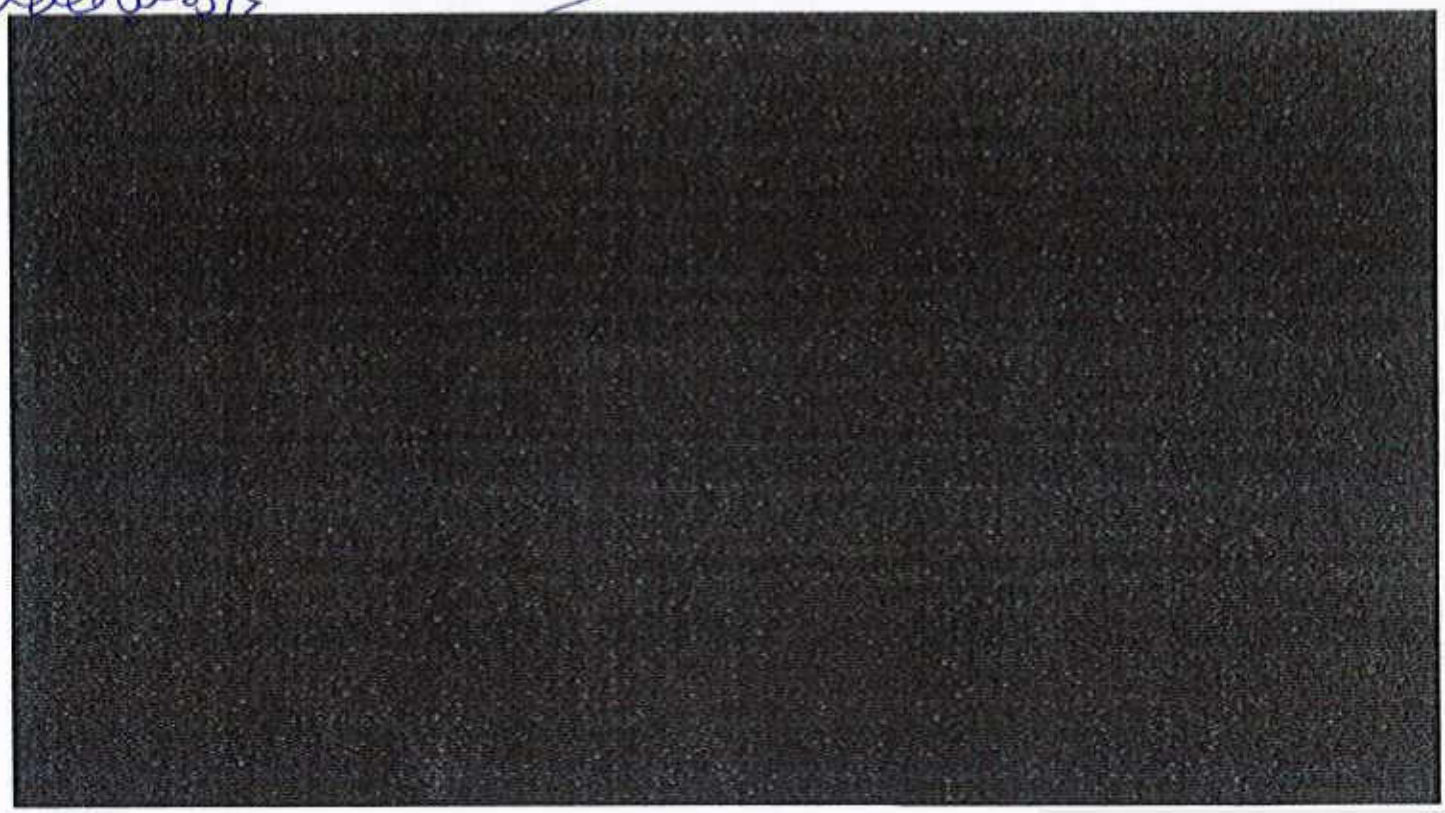
R L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 16/11/2025 G

Name	[illegible]	MRN	5129/C.C1
Age	45 Y.	ID	[illegible]
Menopausal status	pre	Phone	01211000825
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

Bx: IDC

ER:	PR:	HER 2:	Ki67:
+ve 3/3	+ve 3/3	2+ SISH: Amp+	

Sonomammography: Pre-op Rt Breast ca. with Mets to Bone

Mets Workup: Hstry :- 2022 :- Rt Breast ca. T4N0M0 Stage IIIB
NBCS 4 PT2 Tamoxifen 5/2022

R POS: 1 → New Biopsy Rt & Lt Breast
Recurrent

TNM

Recommendations: Rt axilla Recurrence → 2/2024 Rt MLE, Rec adj Taxol, Carbo → Tolad + AI
6/2025:- Met to Bone + Rt Breast Biopsy Cx + PAT; Her 2+ SISH +

MDT OUTCOME & PLAN Rec 4 Taxotere + Herceptin
PET-CT: RD) Rt Breast & axilla LN
Mild Metabolic progression Bone
Stable
5/11/25 SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
In view of mild metabolic progression on taxotere + Herceptin IV. to continue					

Patient Name	: الاستاذه / جيهان عاطف محمد مسعود	Request No.	: 13008554
Patient No.	: 27807088800225	Request Date	: 04/12/2023
Path No.	: 4304/23	Verification Date	: 20/12/2023
Sex	: Female	Financial Category	: ادرام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referring Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Known case of right breast cancer with right axillary lymph nodes.

Nature of Specimen: Tru-cut biopsy, collected at 3/12/2023, 10:10 am.

Gross Examination:
- Two grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
- Histological type: Metastaic mammary carcinoma with lobular features.
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8 /8).
- PR: Strong nuclear reaction in 40% of tumor cells (Allred score 7 /8).
- Her2: modertate complete membrane reaction in >10% of tumor cells (score 2+). (Pending SISH).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:
Known case of right breast cancer with right axillary lymph nodes, Tru-cut biopsy:
Metastaic mammary carcinoma with lobular features.
IPT:
- ER: Positive (score 8 /8).
- PR: Positive (score 7 /8).
- Her2: Equivocal (score 2+) pending SISH.
- Ki67: Positive in 10% of tumor cells.
- For IPT (SISH).

Verified by :
DR.Menna Nabil

CAP
شقة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Cornicha - Misr El Qadeem - Cairo, Egypt

Pathology Report
[[DOCTYPE PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Jihan Atef Mohamed Masoud	Date : 24/07/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 4149/2022	Per. ID: 21008

RIGHT BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had RIGHT CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- right breast UOC focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOC focal parenchymal distortion owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIKADS 2).

Signature Dr. Nehal Hamdy
24/07/2022 2:58 PM

Mammogram
[[DOCTYPE MAMMOGRAM]]

SCAN & LABS

مركز النيل

Name : Gihan Atif Mohammed Masoud
Branch : Dokki

Date : 02-03-2026 01:02
Medical No. : 326070351

٢٠٩٩

ISOTOPIC BONE SCAN REPORT

- **Indication:** 47-year-old female with a history of breast carcinoma, presenting with bone aches. Referred for evaluation of possible osseous metastatic disease.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2--3 hours post intravenous injection.

FINDINGS:

- The scanned skeleton reveals multiple scattered areas of abnormally increased tracer uptake. These lesions involve the skull bones, sternum, both shoulder girdles, scattered bilateral ribs, multiple dorsal and lumbar vertebrae (most notably at L2), and both sacroiliac regions.
- The pattern and distribution of these lesions raise the possibility of metastatic disease.

IMPRESSION:

- Multiple scattered worrisome osseous lesions as described; metastatic disease is considered in this clinical setting, for radiological correlation and clinical follow-up as indicated.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

NILE SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Gihan Atif Mohammed Masoud	Date : 27-10-2025 11:55
Branch : Nasrcity	Medical No. : 235032

Whole body PET/CT EXAMINATION:

Clinical History:

- 47-year-old with history of managed right breast cancer.
- **PET/CT requested for assessment.**

Technique:

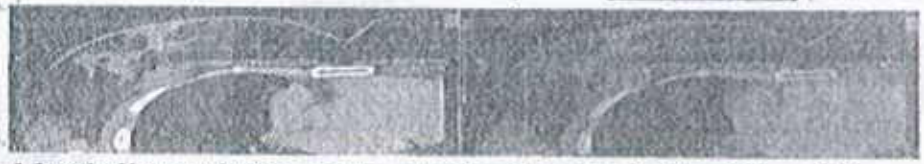
- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis.

Comparison done with PET-CT dated 24-5-2025.

Report

Hepatic reference activity: 3.9

Status post right breast CBS; still seen right breast LOQ parenchymal soft tissue densities with associated nipple retraction currently with SUVmax~2.5 *(was SUVmax~3.5)*



Metabolic resolution of the previously seen right axillary LNs, currently seen measuring about 1.1 cm with no significant metabolic activity *(was SUVmax~2.9)*



Mild metabolic progression of the previously seen multiple predominantly lytic FDG avid osseous lesions are seen involving right scapula, bilateral iliac bones, the spine, left 9th rib and sternum with current highest SUVmax~13.4 at DV12 *(was SUVmax~10.9)*



Head and Neck:

* No hypermetabolic cervical lymphadenopathy could be detected.
* Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
* The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

* No hypermetabolic mediastinal lymphadenopathy.
* No FDG avid lung nodules or masses.
* There are no active pleural abnormalities.
* Physiologic FDG uptake is noted in the heart.

PET Scan
[[DOCTYPE:PET_SCAN]]

Name : Jihan Atef Mohamed Masoud	Date : 24-05-2025 08:00
Branch : Dokki	Medical No. : 110882813
Ref.Physician Prof : MohamedSalamaMohamed	Ins.Company : 2025

Whole body PET/CT EXAMINATION:

Clinical History:

- *47 years old patient diagnosed with managed right breast cancer.*
- ***PET/CT requested for assessment.***

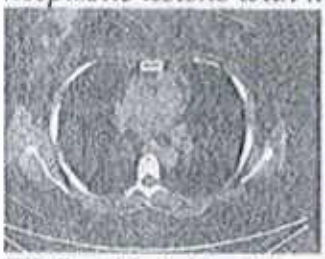
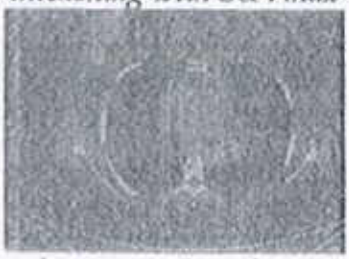
Technique:

- *For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is* ***< 2.5.***

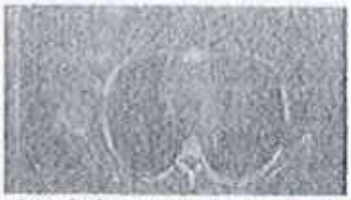
Report:

No previous PET/CT available for comparison

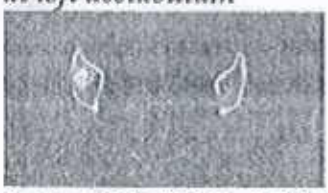
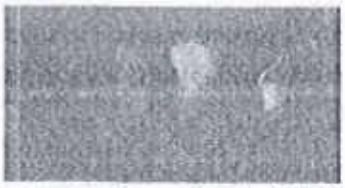
Abnormal FDG uptake is seen over right breast retro areolar and LOQ ill-defined neoplastic lesions with nipple retractions and skin thickening with SUVmax~3.5.

FDG avid right axillary and subpectoral lymph nodes, measuring about 1 cm with SUVmax~2.9.

Multiple FDG avid osseous lesions are seen involving left acetabulum , bilateral iliac bones,LV2,3,4 ,DV12,10,7,6,5 , left 9th rib and sternum with highest SUVmax~10.9 at left acetabulum

Low grade FDG avid mediastinal lymph nodes, measuring about 1.2 cm with SUVmax~1.9 for follow up

Normal tracer uptake over brain, myocardium and renal system.

NILE DIAGNOSTIC CENTER

PET Scan
[[DOCTYPE PET_SCAN]]

Patient's Name:	IHHAN . MMED
Date:	2026-04-28
Referred by:	Prof.Dr.
Hospital:	Gaustave Roussey International Egypt

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

INDICATION:
Right CBS

TECHNIQUE:
- Bilateral MLO and CC views
- Bilateral U/S

REPORT:
- scattered fibroglandular density , (ACR B).
- thickened right breast skin associated with large dense central mass, exhibiting nipple retraction and fungation clinically ,by US, seen involving the area between 8 to 12 oclock, measuring 7 x 4.4 cm, withh associated skin thickening
- no suspicious left cystic or solid lesions
- Left multiple suspicious lymph nodes, the largest measures 15 x 8 mm, with cortical thickness of 4.6 mm
- free supra and infraclavicular nodes

OPINION:
- Normal study of the left breast, (BIRADS 1).
- right postoperative changes with highly suspicious mass for histopathological correlation ,(BIRADS 5).

dr farah elshobaky

Mammogram
[[DOCTYPE MAMMOGRAM]]

٢٧٨٠٧٠٨٨٨٠٠٢٢٥ ٢٠٢٢/٠٥

حاصله على بكالوريوس خدمه اجتماعيه

انثى مسلمة متزوجة

رامى عتر رزق احمد

١٠ البطاقة سارية حتى ٢٠٢٩/٠٥/٢١

٣٠٩٩

جمهورية مصر العربية

بطاقة تحقيق الشخصية

جبهان

عاطف محمد مسعود

ا ش الفارابى خالد بن الوليد

امبابة - الجيزة

٢٧٨٠٧٠٨٨٨٠٠٢٢٥

JJ7389069

National ID - Front & Back
[[DOCTYPE_ID_FRONT_BACK]]

Patient's Name:	JIHAN . MMED
Date:	2026-04-26
Referred by:	Prof.Dr.
Hospital:	Goustave Roussey International Egypt

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

INDICATION:
Right CBS

TECHNIQUE:
- Bilateral MLO and CC views
- Bilateral U/S

REPORT:
- scattered fibroglandular density , (ACR B).
- thickened right breast skin asccociated with large dense central mass exhibiting nipple retraction and fungation clinically .by US, seen involving the area between 8 to 12 oclock, measuring 7 x 4.4 cm. withh associated skin thickening
- no suspicious left cystic or solid lesions
- Left multiple suspicious lymph nodes, the largest measures 15 x8 mm with cortical thickness of 4.6 mm
- free supra and infraclavicular nodes

OPINION:
- Normal study of the left breast, (BIRADS 1).
- right postoperative changes with highly suspicious mass for histopathological correlation ,(BIRADS 5).

dr farah elshobaky

Mammogram
[[DOCTYPE MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Jihan Atef Mohamed Masoud	Date : 26/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 4149/2021	Pat. ID: 21338

TARGETED RIGHT BREST HIGH RESOLUTION ULTRASOUND

Indication:

Patient had Right CBS

Technique

- ultrasound

Report:

- right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- Right axillary tail small anechoic cystic lesion with turbid content measuring about 7x4mm, no internal vascularity seen within.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Right axillary tail benign looking lesion likely fat necrosis for follow up after 3 months (BIRDAS 3)

Signature Dr. Nehal Hamdy

26/03/2025 1:47 PM

Sonography
[[DOCTYPE:SONOGRAPHY]]

Name : Gihan Atif Mohammed Masoud
Branch : Dokki

Date : 02-03-2026 01:02
Medical No. : 326070351

٣٠٩٩

ISOTOPIC BONE SCAN REPORT

- **Indication:** 47-year-old female with a history of breast carcinoma, presenting with bone aches. Referred for evaluation of possible osseous metastatic disease.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection.

FINDINGS:

- The scanned skeleton reveals multiple scattered areas of abnormally increased tracer uptake. These lesions involve the skull bones, sternum, both shoulder girdles, scattered bilateral ribs, multiple dorsal and lumbar vertebrae (most notably at L2), and both sacroiliac regions.
- The pattern and distribution of these lesions raise the possibility of metastatic disease.

IMPRESSION:

- Multiple scattered worrisome osseous lesions as described; metastatic disease is considered in this clinical setting, for radiological correlation and clinical follow-up as indicated.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient's Name:	**GEHAN ATEF MOHAMED MASAEOUD**
Date:	**2026-03-31**
Referred by:	Prof.Dr. CT CAP
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE NON CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

FINDINGS:

Clinically reported breast carcinoma.

Compared to the last study dated 18/9/2025 ,the current study revealed .

(I) Chest:

- **Right breast lesion with nipple retraction .**
- **No appreciable changes regarding the left axillary lymph nodes , the largest measures 18 x 10 mm compared to 16 mm .**
- **No sizable pulmonary nodules , masses or cavitataing lesions .**
- **Still noted enlarged mediastinal lymph node enlargement , the largest at right paratracheal group meaures 16 x 14 mm .**
- **No pleural or pericardial collections seen.**
- **No gross cardiac abnormality.**

(II) Abdomen and pelvis:

- **The liver is enlarged in size showing homogenous CT density no hepatic focal lesions .**
- **Patent and homogenously enhancing portal vein.**
- **No intra or extra hepatic biliary radicles dilatation.**
- **Bilateral renal stones .**
- **Left mild dilatation of left pelvicalcyceal system and left ureter upon lower ureteric stone 8mm with density 650 HU.**

CT Scan
[[DOCTYPE.CT_SCAN]]

جوستاف روسي

Patient's Name:	**GEHAN ATEF MOHAMED MASAEOUD**
Date:	**2026-03-31**
Referred by:	Prof.Dr. **CT CAP**
Hospital:	**Goustave Roussey International Egypt**

- **Normal CT appearance of the spleen , aorta and IVC.**
- **Unremarkable other pelvic organs.**
- **No retro-crural, para-aortic or pelvic lymphadenopathy.**
- **No ascites.**
- **Still noted diffuse scattered sclerotic lesion of axial and appendicular skeleton .**
- **D10 mild vertebral structural collapse .**

OPINION:

- **Overall stationary course of study**

Dr.Reham Khedr, MD

CT Scan
[[DOCTYPE.CT_SCAN]]

Patient's Name:	JIHAN .	MMED
Date:	2026-04-26	
Referred by:	Prof.Dr.	
Hospital:	Goustave Roussey International Egypt	

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

INDICATION:
Right CBS

TECHNIQUE:
- Bilateral MLO and CC views
- Bilateral U/S

REPORT:
- scattered fibroglandular density , (ACR B).
- thickened right breast skin asccociated with large dense central mass exhibiting nipple retraction and fungation clinically .by US, seen involving the area betwéen 8 to 12 oclock, measuring 7 x 4.4 cm, withh associated skin thickening
- no suspicious left cystic or solid lesions
- Left multiple suspicious lymph nodes, the largest measures 15 x8 mm with cortical thickness of 4.6 mm
- free supra and infraclavicular nodes

OPINION:
- Normal study of the left breast, (BIRADS 1).
- right postoperative changes with highly suspicious mass for histopathological correlation ,(BIRADS 5).

dr farah elshobaky

Mammogram
[[DOCTYPE MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Jihan Atef Mohamed Masoud	Date : 26/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 4149/2021	Pat. ID: 21338

TARGETED RIGHT BREST HIGH RESOLUTION ULTRASOUND

Indication:

Patient had Right CBS

Technique

- ultrasound

Report:

- right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- Right axillary tail small anechoic cystic lesion with turbid content measuring about 7x4mm, no internal vascularity seen within.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Right axillary tail benign looking lesion likely fat necrosis for follow up after 3 months (BIRDAS 3)

Signature Dr. Nehal Hamdy

26/03/2025 1:47 PM

Sonography
[[DOCTYPE,SONOGRAPHY]]


جمهورية مصر العربية
بطاقة تحقيق الشخصية
جيهان
عاطف محمد مسعود
١ ش الفارابى-خالد بن الوليد
امبابه - الجيزه
٢٧٨٠٧٠٨٨٨٠٠٢٢٥
JJ7389069



National ID – Back
[[DOCTYPE:ID_BACK]]

Name : Gihan Atif Mohammed Masoud
Branch : Dokki

Date : 02-03-2026 01:02
Medical No. : 326070351

٣٠٩٩

ISOTOPIC BONE SCAN REPORT

- **Indication:** 47-year-old female with a history of breast carcinoma, presenting with bone aches. Referred for evaluation of possible osseous metastatic disease.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection.

FINDINGS:

- The scanned skeleton reveals multiple scattered areas of abnormally increased tracer uptake. These lesions involve the skull bones, sternum, both shoulder girdles, scattered bilateral ribs, multiple dorsal and lumbar vertebrae (most notably at L2), and both sacroiliac regions.
- The pattern and distribution of these lesions raise the possibility of metastatic disease.

IMPRESSION:

- Multiple scattered worrisome osseous lesions as described; metastatic disease is considered in this clinical setting, for radiological correlation and clinical follow-up as indicated.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Gihan Atif Mohammed Masoud	Date : 27-10-2025 11:55
Branch : Nasrcity	Medical No. : 235032

Whole body PET/CT EXAMINATION:

Clinical History:

- 47-year-old with history of managed right breast cancer.
- **PET/CT requested for assessment.**

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis.

Comparison done with PET-CT dated 24-5-2025.

Report

Hepatic reference activity: 3.9

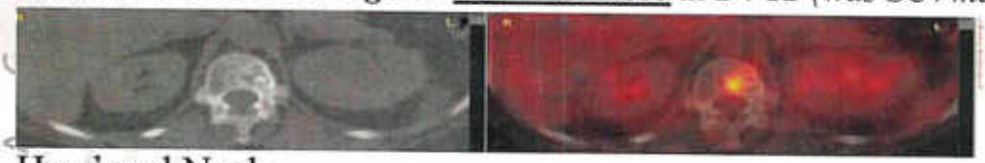
Status post right breast CBS; still seen right breast LOQ parenchymal soft tissue densities with associated nipple retraction currently with SUVmax~2.5 *(was SUVmax~3.5)*



Metabolic resolution of the previously seen right axillary LNs, currently seen measuring about 1.1 cm with no significant metabolic activity *(was SUVmax~2.9)*



Mild metabolic progression of the previously seen multiple predominantly lytic FDG avid osseous lesions are seen involving right scapula, bilateral iliac bones, the spine, left 9th rib and sternum with current highest SUVmax~13.4 at DV12 *(was SUVmax~10.9)*



Head and Neck:

* *No hypermetabolic cervical lymphadenopathy could be detected.*
* *Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.*
* *The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.*

Chest:

* *No hypermetabolic mediastinal lymphadenopathy.*
* *No FDG avid lung nodules or masses.*
* *There are no active pleural abnormalities.*
* *Physiologic FDG uptake is noted in the heart.*

Name : Jihan Atef Mohamed Masoud	**Date : 24-05-2025 08:00**
Branch : Dokki	**Medical No. : 110882813**
Ref.Physician Prof : MohamedSalamaMohamed	**Ins.Company : 2025**

Whole body PET/CT EXAMINATION:

Clinical History:

- *47 years old patient diagnosed with managed right breast cancer.*
- ***PET/CT requested for assessment.***

Technique:

- *For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is **< 2.5**.*

Report:

No previous PET/CT available for comparison

Abnormal FDG uptake is seen over right breast retro areolar and LOQ ill-defined neoplastic lesions with nipple retractions and skin thickening with SUVmax~3.5.



FDG avid right axillary and subpectoral lymph nodes, measuring about 1 cm with SUVmax~2.9.



Multiple FDG avid osseous lesions are seen involving left acetabulum , bilateral iliac bones,LV2,3,4 ,DV12,10,7,6,5 , left 9th rib and sternum with highest SUVmax~10.9 at left acetabulum



Low grade FDG avid mediastinal lymph nodes, measuring about 1.2 cm with SUVmax~1.9 for follow up

Normal tracer uptake over brain, myocardium and renal system.

PET Scan
[[DOCTYPE.PET_SCAN]]

Name : Jihan Atef Mohamed Masoud	**Date : 24-05-2025 08:00**
Branch : Dokki	**Medical No. : 110882813**
Ref.Physician Prof : MohamedSalamaMohamed	**Ins.Company : 2025**

Head and Neck:

* *Normal diffuse brain activity with no hypermetabolic brain lesions.*
* *Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.*
* *Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.*
* *Otherwise, Normal size, enhancement & symmetrical appearance of thyroid glands with no FDG avid lesions detected.*
* *Normal appearance of the neck muscles & vessels.*
* *No hypermetabolic cervical lymphadenopathy detected.*
* *No FDG avid neck soft tissue mass lesions.*

Chest:

* *No FDG avid lung nodules or masses detected.*
* *No metabolically active pericardial or pleural lesions detected.*
* *Physiologic FDG uptake is noted in the heart, showing normal shape.*

Abdomen and Pelvis:

* *Average sized liver with no focal FDG avid lesions.*
* *Average size spleen with no FDG avid focal lesions seen.*
* *Normal size, parenchymal thickness & density of both kidneys with bilateral renal stones , let renal cortical cyst.*
* *Normal appearance of the pancreas, aorta and I.V.C.*
* *No focal FDG avid adrenal lesions.*
* *No hypermetabolic abdominal or pelvic lymphadenopathy.*
* *No FDG avid ascites or abdominal collections noted.*

Conclusion:

PET/CT study showing:

Positive study for FDG avid residual right breast neoplastic lesions with FDG avid ipsilateral axillary nodal and osseous metastatic deposits.

Thank you for referral

Dina Thabet, MSc

Walid Omar, MD.
Prof. of Nuclear Medicine

NILE DIAGNOSTIC CENTER

جوستافروسي انترناشونال مصر

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 9 / 10 / 2025 G

Name	[illegible]	MRN	[illegible]
		ID	٤١٤٩/cc١
Age	45 yrs	Phone	01211000825
Menopausal status	Premenopausal	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	HTN		

Clinical Data

BX: IDC

ER:	PR:	HER 2: (2+)	Ki67:
8/8	8/8	SISH [illegible]	

Sonomammography:

Mets Workup:

[illegible] PET

R L

TNM

Recommendations:

2022 Ruptured Her2ve BC → NACT (AC/TXT) → Rt BCS ypT2 No → PORT → Tamoxifen

11/2023 local recurrence → Rt WLE → save IHC → pseudo adjuvant Taxol/Carbo x18 wk → Zoladex/AI

9/2025 loco-regional recurrence → biopsy triple +ve

MDT OUTCOME & PLAN PET/CT → bone mets → Taxotere/Herceptin x4 → Subjective response of loco-regional recurrence

For MDT to review PET/CT for CT CAP ?? newly developed osteolytic lesion and decide accordingly

Continue txt + Herceptin / 3 Months

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
[illegible]	[illegible]	[illegible]			

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient Name: Na'esa Ahmed Hassan	Date : 09/11/2025
Exam : X-RAY CHEST	Age : 54 Year(s)
Medical ID: 3665/2024	Pat. ID: 93464

Plain X-ray **Chest** (PA view) revealed:

- *Increase broncho-vascular markings*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*
- *Normal thoracic cage*

- Signature Dr . Nermien Ramadan
- 09/11/2025 2:04 PM

GUSTAVE ROUSSY
INTERNATIONAL
ÉGYPTE

Patient Name: Na'esa Ahmed Hassan	Date : 09/11/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 54 Year(s)
Medical ID: 3665/2024	Pat. ID: 93464

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (14cm at Rt MCL) displaying bright homogenous parenchymal echopattern with smooth surface.*
- *No definite focal lesions or intra hepatic billiary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- *Average distention and capacity with no evidence of stones or mud or masses inside. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *average size, homogenous.*

- **Kidneys: -**
  - *Both are of normal size, shape, echogenicity and well differentiated with good corticomedullary differentiation.*
  - *Left kidney shows parapelvical cyst measuring 3.5 x 4.5 cm.*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites.

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

Uterus and adenexa*; no abnormalities could be detected*

Opinion:

Fatty liver

Left renal cyst